TRAUMA STAR · CRITICAL CARE AIR MEDICAL TRANSPORT

Divert & Destination Protocol

Monroe County Fire Rescue — Florida Keys · TS-CCG Clinical Care Guidelines

DRAFT

CATALOG	STATUS	VERSION	REVIEW
TS-OPS-006	DRAFT — pending MD	v0.1 · Jun 2026	Annual

⚠ DRAFT — NOT FOR CLINICAL USE

This protocol is in draft and is not authorized for clinical use until reviewed and signed by the Medical Director (Dr. Gandia). It is written to be read in full by both the Medical Director and the flight crew. Perform within scope and MCFR / Trauma STAR protocol and medical control.

CONTENTS

1 Purpose & Philosophy

The purpose of this protocol is to give the Trauma STAR flight crew a clear, evidence-based framework for deciding **when** to divert a patient from the originally planned receiving facility and **which** alternate facility to choose, while keeping the final decision where it belongs — with the clinicians at the patient's side. The protocol prioritizes rapid stabilization and timely definitive care, patient and crew safety, and disciplined use of the regional trauma system.

RATIONALE

Trauma STAR exists to **reduce the time between a critically ill or injured patient and definitive care** — a trauma surgeon, a neuro-interventionalist, a pediatric intensivist. The aircraft earns its risk and cost precisely by bypassing closer but less-capable hospitals to reach that definitive resource quickly.[1,3]

A divert is, by definition, a decision to *abandon that plan* because continuing it has become more dangerous than the alternative. That is never a trivial decision, and it is rarely a purely algorithmic one: it weighs the patient's current trajectory, the capabilities of the facilities within reach, the time and safety of each option, and the resources on board. This protocol exists to make that weighing **structured and consistent** across crews and shifts — not to remove the clinician from it.[10]

2 Scope

This protocol applies to **all Trauma STAR air medical transports — adult and pediatric, scene response and interfacility transfer**. It governs both in-flight divert decisions and pre-departure changes of destination. It operates within Florida trauma and air-ambulance regulation (Ch. 64J-1 and 64J-2, F.A.C.) and the Monroe County / regional trauma transport plan, and it does not supersede EMTALA obligations or the authority of the Pilot-in-Command over flight safety.

3 Governing Principle — The Decision Rests with Crew Judgment

Every threshold, default, and rule in this protocol is a **decision aid, not a mandate**. The senior medical crew member, in consultation with the Pilot-in-Command on matters of flight safety and with online medical control whenever it can be reached, makes the **final destination decision based on the patient in front of them**. Where this protocol and the crew's real-time clinical assessment diverge, the crew acts in the patient's best interest and documents the reasoning.

RATIONALE

No written algorithm can anticipate every combination of injury pattern, physiologic trajectory, weather, aircraft performance, and shifting facility status that a transport can present. The evidence on transport decision-making is explicit on this point: structured protocols meaningfully **reduce unwarranted variation** in destination decisions, but experienced clinician judgment at the point of care remains essential and irreplaceable.[10,11]

Accordingly, this protocol sets **defaults and guardrails**. It tells the crew what is usually right, what to weigh, and what must be documented — and then trusts the crew to apply it. A protocol that tried to script every divert would either be ignored in the cases that matter most or would force a wrong decision in a case its author never imagined. The crew is expected to follow the framework, to recognize when a patient falls outside it, and to be able to articulate *why* a given destination served the patient best.

⚖ FINAL DECISION — CREW JUDGMENT

If, in the crew's assessment, rigidly following a rule in this protocol would harm the patient, the crew should deviate, act in the patient's interest, contact medical control as soon as practical, and document the rationale. Sound, well-documented judgment is the standard — not mechanical compliance.

4 Definitions & Time Thresholds

TERM	DEFINITION
Divert	A change in destination from the planned receiving facility to an alternate facility during, or immediately prior to, transport.
Closest appropriate facility	The nearest hospital able to provide *immediate* airway management, circulatory support, and emergency intervention for the patient's condition — even if it is not a trauma or pediatric specialty center.
Specialty center	A designated trauma center, pediatric trauma center, burn, stroke/STEMI, or definitive pediatric hospital.
Incremental transport time	The *additional* wheels-up-to-bedside time required to reach the specialty center instead of the closest appropriate facility, including landing-zone, approach, and ground-leg time at each destination.
Small time difference	**≤ 10 minutes by air.**
Large time difference	**> 10 minutes by air** (any increment approaching ~20 minutes is presumptively large).
No medical control	Online medical direction cannot be reached (radio, phone, satellite, telemedicine) within a clinically acceptable timeframe for the patient's condition.

RATIONALE

The **incremental** time — not the absolute flight time — is what matters to a divert decision, because the question is always "what does the patient gain or lose by choosing the farther destination?" Measuring it wheels-up-to-bedside (including the ground leg at each hospital) keeps the comparison honest; a closer helipad with a long ground transfer may not actually be closer to definitive care.[3]

The **10-minute break point is a practical decision aid**, not a physiologic constant. It reflects the general observation that a few extra minutes of well-managed transport are usually tolerable for a patient who is *not* unstable, whereas larger increments meaningfully increase the exposure of a patient whose trajectory is uncertain or worsening. The crew is expected to slide this threshold with the patient: a deteriorating patient may warrant the closer facility even at a small difference, and a rock-stable, well-controlled patient may safely tolerate considerably more than ten minutes to reach the right specialty resource.[4,5]

⚖ **FINAL DECISION — CREW JUDGMENT**

Treat ≤10 / >10 minutes as a starting point for the conversation, not a stopwatch rule. Document the actual times you weighed and why your destination choice served the patient.

5 Divert Activation Criteria

> A divert may be considered whenever continuing to the original destination is reasonably expected to harm the patient, or when the original destination can no longer safely receive or definitively treat the patient. The triggers below are **prompts to deliberate**, not automatic commands to land.

Patient-related triggers

- New or worsening **hemodynamic instability** despite maximal en-route therapy.
- **Respiratory failure or impending arrest** requiring an intervention that cannot be safely performed in continued flight.
- **Cardiac arrest or peri-arrest** occurring in flight.
- **Rapid neurologic deterioration** — falling GCS, status epilepticus, signs of herniation.
- Any condition in which continued flight to the original destination is reasonably expected to cause **imminent decompensation or death.**

System / destination-related triggers

- Original destination declares **trauma divert / bypass or internal disaster** and cannot safely accept the patient.
- Loss of a **critical capability** at the receiving facility (CT, OR, ICU, blood, pediatric coverage) that would significantly delay definitive care.
- **Weather, airspace, or aircraft / mechanical** issues that make continued flight to the original destination unsafe.

RATIONALE

These triggers fall into two families with very different logic. **Patient triggers** ask whether the patient can survive the planned trip; **system triggers** ask whether the planned destination is still able to deliver the care the trip was for. Either can justify a divert, and they are weighed differently — a patient trigger usually points toward the *closest* capable facility, while a system trigger usually points toward the *next-best specialty* facility.[4,5,6]

A facility's own divert request is informative but not binding. Best-practice trauma-system policy explicitly preserves the crew's authority to **decline a divert and deliver to that facility anyway** when the patient has an uncontrollable, immediately life-threatening problem and diverting further would reasonably increase morbidity or mortality.[5,6]

⚖ FINAL DECISION — CREW JUDGMENT

A trigger means "stop and think," not "land now." The crew decides whether the trigger, in this patient, actually changes the safest destination.

6 Destination Decision Framework

When a divert is under consideration, the crew (1) **assesses** the patient — ABCD, current interventions, and anticipated needs over the next 10–15 minutes; (2) **compares** the time and capability of the original specialty destination against the nearest appropriate facility; and (3) **contacts medical control** as soon as practical for a structured report and destination confirmation. The following are the usual right answers; the crew may depart from them for documented cause.

1. **Unstable or arresting patient → closest appropriate facility** for immediate resuscitation, regardless of incremental time or specialty designation. Secondary transfer follows once stabilized.
2. **Stable but high-risk, small time difference (≤ 10 min) → continue to the original specialty center**, unless it is on divert or has lost a needed capability.
3. **Stable but high-risk, large time difference (> 10 min) with an uncertain or worsening trajectory → closest appropriate facility** for stabilization, then secondary transfer.
4. **Imminent airway failure** favors the closest facility at which a definitive airway can be obtained quickly.

RATIONALE

The organizing idea is the **closest appropriate facility**. For a patient who cannot reliably be kept alive to the specialty center, the nearest hospital that can actually resuscitate is — for that patient, at that moment — the appropriate destination, even if it is "only" a community ED. The evidence-based air-medical trauma guideline supports exactly this: use the closest appropriate facility when instability makes a longer transport unsafe, and otherwise minimize total out-of-hospital time on the way to definitive care.[3]

Note that the two families of trigger from Section 5 resolve differently here. Instability drives the decision *toward* the nearest capable facility (rule 1 and 3); a stable patient whose only issue is reaching the ideal specialty resource is usually best served by *continuing* when the extra time is small (rule 2). The framework is therefore not "divert vs. don't" — it is a continuous trade-off between the value of definitive capability and the cost of additional out-of-hospital time, judged against the patient's trajectory.[3,4]

7 Scene Calls vs. Interfacility Transfers

The structure of a divert decision differs by mission type, and the crew must know which logic applies before launch.

7.1 Scene Calls

On a scene response there is no sending facility and no accepted transfer plan. The crew operates under Trauma STAR standing orders and field trauma-triage criteria, frequently before online medical control is established. The only destination options are **forward** — the closest appropriate facility versus continuing to the planned specialty center.

RATIONALE

Because there is no facility to "return" to and often no physician yet on the line, scene calls grant the crew the **broadest standing-order autonomy** of any mission type. That autonomy is paired with the responsibility to document the field-triage criteria met and the basis for the destination chosen.[4,7]

7.2 Interfacility Transfers (IFT)

On an IFT the patient is already at a sending facility and has been accepted by a receiving facility and physician under an EMTALA-compliant transfer. This adds a third option that a scene call does not have — **returning to the sending facility** — and imposes a pre-departure duty to ensure the patient is adequately stabilized before launch.

IFT — THREE OPTIONS, STABILIZE FIRST

Pre-departure: do not launch an unstabilized patient whom the sending facility can still meaningfully stabilize (its OR, a definitive airway, immediately available blood). **En route, compare wheels-up-to-bedside time to (a)** the sending facility, **(b)** the accepting facility, and **(c)** the closest appropriate facility, and choose whichever delivers temporizing or definitive care fastest for the actual problem. A receiving physician's acceptance **does not bind the crew** to fly past a closer life-saving option. Notify both the sending and accepting physicians of any divert.

RATIONALE

The sending physician retains medico-legal responsibility for the patient until care is formally assumed, and the transfer was a considered, physician-to-physician decision — so an IFT divert is a departure from an *accepted plan* and must be communicated back to both ends. The return-to-sender option exists because deterioration early in transport often occurs while the aircraft is still near a hospital that already knows the patient and was actively working the problem.[5,8]

The pre-departure stabilization duty reflects a simple truth: the worst place to manage a deteriorating airway or uncontrolled hemorrhage is a moving aircraft. If the sending facility can fix or temporize the problem before liftoff, that is almost always safer than launching and hoping.[3,9]

FACTOR	SCENE CALL	INTERFACILITY TRANSFER
Sending facility	None	Yes — "return to sender" possible early in transport
Authority basis	Standing orders + field triage	Sending order + acceptance + standing orders for emergencies
Divert options	Closest appropriate vs. continue	Return to sender vs. continue vs. closest appropriate
Medical control	Often not yet established	Sending physician until handoff; transport MD online
Pre-departure check	Minimize on-scene time	Stabilize at sender first — do not fly the uncontrolled

8 Pediatric-Specific Guidance

For pediatric Trauma STAR patients the closest-appropriate-facility principle is applied even more decisively, because timely stabilization supersedes the ideal specialty destination when delay increases risk.

- **Cardiac arrest or peri-arrest → the nearest hospital capable of pediatric resuscitation**, regardless of trauma designation; secondary transfer follows ROSC or adequate stabilization.
- Still-perfusing, severely injured child for whom pediatric expertise is critical: if the time difference is **small and the child is tolerating** the leg, the pediatric center is reasonable; if it is **large or the trajectory is worsening**, default to the closest appropriate facility, then transfer.
- Delivery of a pediatric arrest to a community or critical-access ED is a **resuscitation and stabilization stop only**; transfer to the designated pediatric center (**Nicklaus Children's**) must begin as soon as ROSC or stabilization is achieved.

RATIONALE

Critically ill children deteriorate quickly and their physiology punishes delay, so the cost of carrying an unstable child past a hospital that could resuscitate now is higher than in adults. The AAP transport guidelines and contemporary review stress regionalized pediatric care *and* the principle that rapid stabilization can take precedence over reaching the ideal specialty center when delay would increase risk.[1,2,9]

The designated pediatric definitive center for Trauma STAR is **Nicklaus Children's Hospital**; Ryder/Holtz provides pediatric surgical trauma capability. These remain the goal destinations for a stable child — the community-ED stop is a bridge to them, never the endpoint.[1,2]

9 Stabilization Stop vs. Continue

Because Trauma STAR exists to bypass community EDs, the **default is to continue** to definitive care. A stop at a community or critical-access ED is the exception, justified only when the crew judges that all three of the following are true.

⛔ **STOP ONLY IF ALL THREE HOLD**

1. There is an **immediate in-flight threat to life that the crew cannot control** in the aircraft — a failed airway, an inability to oxygenate or ventilate, or a sustained arrest requiring ongoing CPR — **and**
2. The closest facility can **plausibly fix or meaningfully temporize** that specific problem — a definitive or surgical airway, blood, chest decompression, or a code run on a stable platform — **and**
3. The **incremental time to the specialty center is large enough** that continuing would reasonably cause death or arrest en route.

✓ CONTINUE WHEN

The patient is being **maintained**, even if critically ill; or the threat is **surgical bleeding** that the closest ED cannot operate on (stopping adds time without adding the fix); or the **time difference is small** and the patient is tolerating transport.

RATIONALE

The pivotal, often-missed fact is that **a moving helicopter cabin is a poor platform for resuscitation**. Chest-compression quality — the single strongest modifiable predictor of return of spontaneous circulation — degrades badly against vibration, in a confined space, with a restrained provider over a restrained patient. For a patient in true arrest, getting onto a flat, stable surface with a full team is frequently worth more than any specialty designation, which is the core reason a stabilization stop can be right.[3]

But a stop only helps if the receiving facility can address the *actual* problem. Landing a patient who is exsanguinating from a torso injury at a hospital with no trauma surgeon and no OR simply adds time before the only thing that will save them — the surgeon. That is why the test is three-pronged: an uncontrollable threat, a facility that can fix *it*, and a time saving large enough to matter. Remove any one prong and the right answer is usually to continue.[3,5]

These prongs are deliberately written as **clinical judgments** — "cannot control," "plausibly fix," "large enough" — because only the crew at the bedside can assess them in real time.

INTUBATED-DETERIORATION CAVEAT (DOPES)

A deteriorating *intubated* patient is a **D**isplacement · **O**bstruction · **P**neumothorax · **E**quipment/oxygen · **S**tacking problem until proven otherwise. Most of these are correctable by the crew in the aircraft — reconfirm $ETCO_2$ and tube placement, suction, disconnect and hand-ventilate, needle-decompress. **Correct and continue** unless the problem is genuinely uncontrollable in flight.

⚖ FINAL DECISION — CREW JUDGMENT

Prongs 1–3 are judgments, not checkboxes. If you stop, be able to say what you could not control, what the receiving ED could do about it, and how much time it saved. If you continue through a rough patch, be able to say how you kept the patient safe.

10 Mechanical CPR (LUCAS) — Pediatric Limitation

Mechanical CPR is restricted to patients who meet the device's adult size limits. It **must not be used on infants or small children**.

⛔ **LUCAS IS ADULTS-ONLY — SIZE-GATED, NOT AGE-GATED**

Fit limits (LUCAS 3): chest height **170–303 mm**, chest width ≤ 449 mm; no weight limit. A patient below the size floor causes the device to **self-reject** — three fast beeps, and it will not enter ACTIVE mode. Do not attempt to defeat this. Only a **large adolescent with an adult-sized chest** who physically fits the device may be managed as an adult; for all smaller children, perform **manual PALS compressions.**

RATIONALE

This limitation has a direct bearing on the stabilization-stop decision in Section 9. The reason a moving cabin is a poor CPR platform can, for an adult, be mitigated by a mechanical compression device that delivers consistent compressions in flight. **That mitigation is unavailable for a small child**, because the LUCAS is an adult device that is physically incompatible with a pediatric chest and is not indicated for pediatric use.[12]

The practical consequence: a **small child in arrest cannot receive high-quality compressions in the aircraft by any means**, which strengthens the case for diverting a pediatric arrest to the nearest capable ED for resuscitation on a stable platform. Conversely, for a large adolescent who fits the device, mechanical CPR may make continuing to definitive care more defensible. Confirm the limits against the instructions-for-use of the specific LUCAS model carried on the ship.[12]

11 Authority & No Medical Control

11.1 Authority to Divert

- **Any crew member** may initiate an urgent divert request if continued flight presents an immediate risk to patient or crew.
- The **senior medical crew member (RN / Paramedic)** makes the clinical destination decision when medical control is required but cannot be reached.
- **Medical control (online physician)** confirms the divert destination when available and required by regulation.
- The **Pilot-in-Command** holds final authority over all aviation-safety matters and selects the safest alternate landing site with crew clinical input.

11.2 No Medical Control Available

> When attempts to reach medical control are unsuccessful or unacceptably delayed **and** the patient faces an imminent threat to life or is deteriorating, the crew is authorized under standing orders to **divert to the closest appropriate facility without prior physician approval** when further delay would jeopardize survival or crew safety.

RATIONALE

Air-medical medical-direction frameworks anticipate exactly this situation: they delegate standing orders and expect crews to act on protocol and clinical judgment when real-time medical control is unavailable, documenting the attempts and the rationale.[13,8] For Trauma STAR the provision is not theoretical — long over-water and remote legs in the Keys make lost communications a foreseeable, recurring reality rather than an edge case.

CREW RESPONSIBILITIES UNDER THE NO-CONTROL PROVISION

Document every failed attempt to reach medical control (times and methods); state explicitly in the record that the divert was made under the “no medical control” provision; and notify both the original and receiving facilities as soon as communication is restored.

12 Operations, Documentation & Quality Review

12.1 Operational steps during a divert

1. Pilot selects the nearest appropriate landing zone for the new destination.
2. Medical crew intensifies resuscitation and prepares a concise handoff — mechanism, injuries, treatments, and response.
3. Communications / Dispatch notifies the original destination of the divert and the receiving facility with ETA and acuity.
4. Crew documents the time and reason for divert, patient condition before and after, medical-control involvement, and the destination rationale.

12.2 Documentation & QA/QI

Every divert is logged with its reasons, a narrative of the clinical and operational decision-making, medical-control contact (or a documented failure to reach it), and the outcome at the receiving facility. The Trauma STAR QA/QI Committee reviews **all** divert cases — and specifically flags **all “no medical control” cases** — at least quarterly, and the Medical Director periodically reviews them to confirm that crew judgment is consistent with best practice and to refine education and protocol.

RATIONALE

Because this protocol deliberately leaves room for judgment, the **quality-review loop is what keeps that judgment calibrated**. Reviewing the actual decisions — including the ones that deviated from the defaults — is how the program distinguishes sound clinical reasoning from drift, feeds real cases back into training, and gives the Medical Director assurance that delegated authority is being exercised well. Trauma-system standards require this tracking and continuous performance improvement.[14,4]

A Hospital Capability Chart

Official regional capability chart — dated 6/12/2026. Blank = service not provided. Use the framework above to select a destination from this matrix, and always verify real-time divert status with medical control / dispatch.

FACILITY	ADDRESS	GENERAL		OB	TRAUMA CENTER			STROKE	STEMI	HAZMAT / RAD	HYPER-BARIC	HELI-PORT	MCI SURGE		
		ADULT	PEDI		ADULT	PEDI	BURN						R	Y	G
MIAMI-DADE COUNTY FACILITIES															
Baptist	8900 North Kendall Drive, Miami	YES	YES	YES				CSC	YES	HAZ/RAD		YES	34	52	87
Baptist Doctors	5000 University Drive, Coral Gables	YES								HAZ/RAD			11	17	28
Baptist Doral	9500 NW 58 Street, Doral	YES											2	5	10
Baptist Homestead	975 Baptist Way, Homestead	YES	YES	YES					YES	HAZ/RAD		YES	4	5	10
Baptist South Miami	6200 SW 73 Street, South Miami	YES		YES				PSC	YES	HAZ/RAD			5	10	20
Baptist West Kendall	9555 SW 162 Avenue, Miami	YES		YES				PSC		HAZ/RAD			9	14	23
Baptist-Coral Way West FSED	14591 SW 26 Street, Miami	YES											2	5	10
Baptist-Country Walk FSED	14150 SW 136 Street, Miami	YES											2	5	10
Baptist-Miami Lakes FSED	15200 NW 77 Court, Miami Lakes	YES											2	5	10
Baptist-Palmetto Bay FSED	8750 SW 144 Street, Miami	YES											2	5	10
Coral Gables HSA	3100 Douglas Road, Coral Gables	YES	YES					PSC	YES	HAZ/RAD			5	7	11
HCA FL Aventura	20900 Biscayne Blvd, Aventura	YES	YES		YES			CSC	YES	HAZ/RAD		YES	7	11	18
HCA FL Kendall	11750 Bird Road, Miami	YES	YES	YES	YES	YES	YES	CSC	YES	HAZ/RAD		YES	15	22	36

FACILITY	ADDRESS	GENERAL		OB	TRAUMA CENTER			STROKE	STEMI	HAZMAT / RAD	HYPER-BARIC	HELI-PORT	MCI SURGE		
		ADULT	PEDI		ADULT	PEDI	BURN						R	Y	G
HCA FL Mercy	3663 South Miami Avenue, Miami	YES	YES	YES				CSC	YES	HAZ/RAD	YES	YES	8	12	20
HCA FL-Cutler Bay FSED	19605 South Dixie Highway, Cutler Bay	YES	YES							HAZ/RAD			2	5	10
HCA FL-Doral FSED	10915 NW 41 Street, Doral	YES	YES							HAZ/RAD			2	5	10
HCA FL-Palm Lakes FSED	18000 NW 57 Avenue, Hialeah	YES	YES							HAZ/RAD			2	5	10
HCA FL-South Dade FSED	14190 SW 288 Street, Homestead	YES	YES							HAZ/RAD			2	5	10
HCA FL-Town & Country FSED	11800 Sherry Lane, Miami	YES	YES							HAZ/RAD			2	5	10
Hialeah HSA	651 East 25 Street, Hialeah	YES						PSC		HAZ/RAD			5	10	15
Jackson Memorial Ryder Trauma	1026 NW 19 Street, Miami 1800 NW 10 Avenue, Miami	YES	YES	YES	YES	YES	YES	CSC	YES	HAZ/RAD		YES	42	63	105
Jackson North Medical Center	160 NW 170 Street, North Miami Beach	YES	YES	YES				PSC	YES	HAZ/RAD			5	10	15
Jackson South Medical Center	9333 SW 152 Street, Miami	YES	YES		YES			PSC	YES	HAZ		YES	10	15	25
Jackson West Medical Center	2801 NW 79 Avenue, Doral	YES	YES						YES	HAZ			4	6	10
Larkin Palm Springs	1475 West 49 Place, Hialeah	YES						CSC	YES	HAZ/RAD			10	15	25
Larkin South Miami	7031 SW 62 Avenue, South Miami	YES							YES	RAD			2	5	10
Mount Sinai Medical Center	4300 Alton Road, Miami Beach	YES	YES	YES				CSC	YES	HAZ/RAD		YES	8	15	20
Mount Sinai-Aventura FSED	2845 NE 199 Street, Aventura	YES	YES							HAZ/RAD			2	5	10
Mount Sinai-Hialeah FSED	6050 West 20 Avenue, Hialeah	YES	YES							HAZ/RAD			2	5	10
Nicklaus Children's	3100 SW 62 Avenue, Miami		YES			YES				HAZ/RAD		YES	7	10	17
North Shore Medical Center HSA	1100 NW 95 Street, Miami	YES	YES							HAZ			5	17	33
Palmetto General HSA	2001 West 68 Street, Hialeah	YES	YES	YES				CSC	YES	HAZ/RAD			15	22	37

FACILITY	ADDRESS	GENERAL		OB	TRAUMA CENTER			STROKE	STEMI	HAZMAT / RAD	HYPER-BARIC	HELI-PORT	MCI SURGE		
		ADULT	PEDI		ADULT	PEDI	BURN						R	Y	G
University of Miami	1400 NW 12 Avenue, Miami	YES	YES					PSC	YES	HAZ/RAD			19	28	47
Veterans Medical Center	1201 NW 16 Street, Miami	YES						PSC					2	5	10
Westchester-Coral West	2500 SW 75 Avenue, Miami	YES								HAZ			4	6	10
FLORIDA KEYS HOSPITALS															
Lower Keys Medical Center	5900 College Rd, Key West											YES			
Fishermen's Community Hospital	3301 Overseas Hwy, Marathon											YES			
Mariners Hospital	91500 Overseas Hwy, Tavernier											YES			

Blank Space = Service not Provided

BURN = Burn Unit Center

CSC = Comprehensive Stroke Center

FSED = Free Standing Emergency Department

HAZ = HAZMAT Capable

MCI = Mass Casualty Incident: Use this chart only for MCI Level 3 or higher. For trauma MCIs at Levels 1-2, transport all Red-tagged patients to designated Trauma Centers.

PSC = Primary Stroke Center

RAD = Radioactive Capable

STEMI = ST Elevation Myocardial Infarction

Trauma Center = Brain and Spinal Cord Acute Care Center

Note: If MERCY is off line for hyperbaric, St. Mary Hospital, West Palm Beach or Mariners Hospital, Tavernier can be used.

B Worked Examples

EXAMPLE 1 — PEDIATRIC ARREST IN FLIGHT (SCENE/IFT, KEY WEST → NICKLAUS)

Situation. Intubated child, Lower Keys Medical Center (Key West) bound for Nicklaus Children's. Fifteen minutes out (≈ overhead Marathon) the child goes into full cardiopulmonary arrest. Beneath the aircraft is Fishermen's Community Hospital (ED, helipad) — minutes away; Nicklaus is ~35–40 minutes on.

Reasoning & decision — divert to Fishermen's. All three Section 9 prongs hold: a sustained arrest cannot be effectively managed in a moving cabin and the child is too small for LUCAS (Section 10); Fishermen's can run a full PALS code on a stable platform; and the incremental time to Nicklaus is large. The airway is already secured, so a community ED is not a downgrade for the arrest itself. The crew lands, resuscitates, and — on ROSC — re-launches to Nicklaus. *The protocol points here, but it is the crew's read of "uncontrollable in this cabin" that makes the call.*

EXAMPLE 2 — MAINTAINED ADULT, SMALL TIME PENALTY (CONTINUE)

Situation. Adult with a severe but controlled TBI, intubated, sedated, normotensive, $ETCO_2$ on target, en route to a trauma center 12 minutes beyond the nearest community ED.

Reasoning & decision — continue. The patient is being maintained; no Section 9 prong is met. The community ED cannot offer the neurosurgical capability the patient needs, so stopping would add time without adding the fix. The small-to-moderate incremental time is justified by the definitive capability at the trauma center. The crew continues, with a low threshold to reassess if the trajectory changes.

EXAMPLE 3 — IFT, EARLY DETERIORATION (RETURN TO SENDER)

Situation. IFT of an adult with a surgical abdomen accepted by a mainland trauma center. Five minutes after liftoff the patient's pressure collapses and bleeding appears uncontrolled; the sending hospital — which has an OR and was already typing blood — is the closest facility.

Reasoning & decision — return to sender. Deterioration is early, the threat is surgical bleeding, and the sending facility can operate; that combination favors the one nearby hospital that can actually fix the problem. The crew returns, notifies both physicians, and documents the departure from the accepted plan.

Approval & Sign-Off

Not authorized for clinical use until signed by the Medical Director.

Medical Director	EMS Director / Trauma STAR Program
Dr. Gandia — signature	Signature
Date	Date

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